

Dear Unite Us hiring team,

I'm the Lead Data Engineer at Pharmacists Mutual, where I set the architectural direction for our move off on-prem SQL Server and SSIS onto a medallion lakehouse on Azure and Microsoft Fabric, and I lead the small team that owns the company's analytical reporting. I want my next stop to have a more obvious, observable effect. Your post lines up with that - a network tying healthcare/government/community organizations together, so that people actually get the care they need.

Over ten years I've owned an ETL and data warehouse stack end to end. I rebuilt enterprise reporting from scratch after our vendor collapsed, integrated Ataccama MDM so data flows through governed master records instead of straight from source, and reported architecture decisions directly to company leadership as the sole data strategist. I've since grown into leading a team. I recruited and trained a new senior data engineer - we call him "Second Dave", since we already had one. I led the planning and implementation that turned two federal/state reporting systems - systems that used to live in "First Dave's" head - into documented, team-owned products.

My personal project, Gutenberg-Fingerprint, is closer to your stack: a nightly CDC pipeline in Postgres, dbt Core, and Dagster, with SCD2 snapshots, source-freshness checks, a full test suite, all provisioned with Terraform, all running on a \$5/month VPS. With all due candor, it's not Snowflake and Airflow. Same pattern though, warehouse plus dbt plus orchestrated ETL, just solo and cheap. SQL Server and dbt transfer fast, and I pick up new warehouse engines quickly. (Personally, I think learning new things is what separates human beings from oxen.)

Reach out if any of this experience and enthusiasm sounds like it would transfer to the Unite Us culture.

All the best,
Sander VanWilligen